QED

By Peter Parnel

RICHARD: The thing about solving a problem – in science or in life – is that you have to think a problem through rationally, even if you disagree with your outcome emotionally. See, if your intuition doesn't accept what your reasoning concludes, that's okay – you're entitled to your feelings. But at least you've gotten yourself to ***think*** about it.

INTRODUCTION

Richard Feynman, a physics professor at Cornell University, is a man faced with a choice. He has to decide whether or not he will undergo surgery that could extend his life or end it. He is also tormented by memories of his wife and the desire to believe in something more than science.

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RICHARD: When I was a kid growing up in Far Rockaway, I loved to solve puzzles. In high school, during first period, a guy would come up to me with a puzzle in geometry. I wouldn't stop until I figured the damn thing out. Then, during the day, other guys would come to me with the same problem, and I'd do it for them in a flash. So for one guy, it took me twenty minutes, while there were five guys who thought I was a super-genius.

So I got a fancy reputation. And it might not have been all that deserved, but what did I care?!

Then, later, at Cornell, I was still solving puzzles! Only they were bigger! I tried solving the infinities of quantum electrodynamics, QED. I'd worked on it since graduate school. But the thing is, I solved it my own way, by creating my own system! Because I didn't trust the experts who'd solved everything before me, I resolved everything they did, as if it was the first time. "Know How To Solve Every Problem That's Already Been Solved" – that's one of my mottoes – because, "what I Cannot Create, I do Not Understand."

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Sometimes you get so caught up with solving a puzzle, you loose sight of the reality of the world outside the puzzle. Like with this cancer I've got. It's the oddest thing – it doesn't metastasize. It just spreads. It just grows like a giant tree.

The first cancer I got, in my abdomen, got removed about four years ago along with one of my kidneys. They thought they got it all. Then it shoed up in a crazy pattern around my intestines. Dr. Hackett told me he wanted to cut out as much of it as he possibly could. He said he wouldn't stop until he saw the bottom of the operating table underneath.

Hackett told me he never expected me to make it through that first surgery – and I've had too more since!

I was just reading here . . . if I do this operation, I'll probably lose my one good kidney and be on dialysis for the rest of my life . . . Mmmmm . . . You know, I'm wondering, what am I looking at if we *don't* go in? . . . Six months? A year? What's the risk involved while I'm on the table? . . . And what about post-operatively? What are my chances of dying or living?

What should I do? Should I do this? How can I answer such a question? "If I do this, what will happen?" See, that's usually a scientific question: The answer is always: "Try it and see." But with this, I only have one chance.

All of this, and they can't even tell me how much time the operation would buy me . . . . Just for some kind of hope. Hope! What good is hope? Why should I console myself with crazy, mythical thinking?

My wife, hadn't.

When she first go sick, we made a pact that we must speak honestly with each other and look at everything directly. Her doctors told me she had Hodgkin's disease – I told her she had glandular fever, a much less serious disorder. I lied to her. And she believed me, for a while. When she suspected something was wrong, she asked me again. And I told her the truth.

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And she thanked me for it. She was better at facing what was happening to her than I was . . . I clung to false hope. I had to! How could I not . . . ?

In the last days, at the hospital . . . Arlene was very weak. She didn't seem to know what was happening . . . She stared straight ahead most of the time, and she was trying hard to breathe. Every once in a while her breathing would stop, and then it would go on again . . . . I felt strange, sort of calm in a way. As I watched her, I kept imagining all the things going on physiologically – the lungs not getting enough oxygen into her blood, making the brain fogged out and the hear weaker which made the breathing more difficult. I kept expecting some sort of avalanche, some dramatic collapse . . . but it didn't appear that way at all . . . she just got foggier . . . and her breathing became more shallow . . . until there was only one very small one.

It was several months before I could react emotionally to her death. Not that I didn't feel it deeply. But it wasn't sinking in, until I was walking past a department store with dresses in the window, and I saw a pretty dress, and I thought, she would like that. And then it hit me.

I don't know . . . suddenly it all came out . . . it was too much for me . . .

A couple years after she died, I wrote Arlene a letter. I haven't looked at it for . . . I haven't looked at it in over forty years.

> *"Arlene, I adore you . . . it is such a terribly long time since I last wrote you, but I know you'll understand, because you know how I am – stubborn and realistic – and I thought there was no sense in writing. But now I know, my darling wife, that it is right to do what I have delayed in doing. I want to tell you I love you. I always will love you.*

You, dead, are so much better than anyone else alive. (I love my wife. My wife is dead)

P.S. Please excuse my not mailing this, but I don't know your new address

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I've been dreaming about her again lately.

I'm not going to do the operation. It's time to simply let go. Remember what the old folktale says: "Don't cry. Don't be sad. All men must die, and you are not the first."

SOURCE INFORMATION

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AWARD HISTORY

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